DOCVAIL

Real-Time Doctor Availability for Every Indian

Govt + Private Hospitals

Rural + Urban Areas

"No more wasted trips. Know before you go."

Startup Pitch Deck

The Problem

Millions of Indians waste time & money visiting hospitals with no doctor available

Wasted Trips

Patients, especially in rural areas, travel hours to hospitals only to find their desired doctor is absent. No way to check availability beforehand. This is the single biggest pain point in Indian outpatient healthcare.

Rural Healthcare Desert

India has 1 govt doctor per 10,189 people in rural areas (WHO recommends 1:1,000). Doctors split time across multiple facilities with zero visibility to patients about their schedule.

No Real-Time Information

Neither govt nor private hospitals publish real-time OPD schedules online. Patients rely entirely on word-of-mouth, phone calls that go unanswered, or repeated visits to check.

Financial Burden on the Poor

Each wasted trip costsINR 200-1,000+ in travel, lost wages, and missed work. For daily-wage earners, this can mean a full day's income lost — devastating for families already struggling.

The Scale of the Problem

45%
of rural patients

travel >10km
for basic OPD

63%
ofwasted hospital
visitsdue todoctor
unavailability

1 : 10,189
doctor-to-patient
ratio in rural
India (govt)

INR 200-1K+
lost per wasted
trip (travel+
lost wages)

WHY THIS MATTERS

- India's public health infrastructure serves 70%+ of rural population, but lacks any digital availability system for patients.
- Private clinics and hospitals (60% of urban healthcare) also have no standardized doctor availability display for patients.
- Patients with chronic conditions (diabetes, BP, heart disease) need regular specialist visits — each wasted trip compounds the problem.
- Women, elderly, and disabled patients face disproportionate hardship from unnecessary travel — safety risks and physical strain.
- An estimated 63% of first-time hospital visits in rural India result in the patient not finding the doctor they came for.

The Solution: Docvail

A real-time Doctor availability platform for ALL hospitals — Government + Private

Patient App

- Search doctors by specialty, location, hospital type
- See real-time availability status of any doctor
- Book appointment and pay consultation fee online
- Get instant confirmation with timing and directions
- Rate and review doctors after visit

Doctor Dashboard

- Set daily OPD timings (morning/evening/custom)
- One-tap 'Available Now' toggle for instant status
- Define appointment slots (15/30/60 min durations)
- View & manage all upcoming booked patients
- Receive 80% of every consultation fee earned

Hospital Integration

- Easy onboarding for private clinics & hospitals
- Govt hospital data via authorized API access
- Multi-doctor schedule view for admin staff
- OPD queue management and wait-time display
- Analytics dashboard for hospital administration

Govt Data Layer

- Authorized real-time access to govt hospital rosters
- ABDM / ABHA health ID ecosystem integration
- Live data display on Docvail for all users
- District-level dashboards for health officers
- Alignment with Digital Health Mission APIs

"Know before you go" — Check doctor availability from home, save time, save money.

How It Works

DOCTOR SIDE

1. Registers on Docvail platform
2. Sets daily OPD timings & slots
3. Marks availability (1-tap toggle)
4. Receives booked appointments
5. Checks patients, earns 80% fee

PATIENT SIDE

1. Opens Docvail app or website
2. Searches doctor by specialty & area
3. Sees real-time availability status
4. Books appointment & pays fee
5. Visits doctor at confirmed time

At Booking: Patient pays consultation fee

80% to Doctor | 20% toDocvail (commission)

Revenue Model: 80/20 Commission

Simple, scalable, and aligned with doctor success

PRIMARY REVENUE: APPOINTMENT COMMISSION

Patient books appointment on Docvail >> Pays consultation fee >> Auto-split

Doctor receives 80%

Docvail retains 20%

Example: *Fee: INR 300* *Doctor: INR 400* *Docvail: INR 60*

SECONDARY REVENUE (FUTURE)

- Premium / featured doctor profiles (paid listing)
- Government data integration contracts (B2G)
- Hospital analytics dashboard subscription
- Sponsored health content and promotions

WHY THIS MODEL WORKS

- Zero friction for doctors — free to join, earn from day 1
- Revenue grows linearly with every booking
- No inventory risk — pure marketplace commission
- Aligned incentives — more patients = more for everyone

Unit Economics

Metric	Conservative	Moderate	Aggressive
Avg Consultation Fee	INR 300	INR 500	INR 700
Docvail Commission (20%)	INR 60	INR 100	INR 140
Daily Bookings (Y1)	200	400	800
Monthly Revenue	INR 3.6 L	INR 12 L	INR 33.6 L
Annual Revenue (Y1)	**INR 43.2 L**	**INR 1.44 Cr**	**INR 4.03 Cr**

3-YEAR PROJECTION (MODERATE SCENARIO)

	Year 1	Year 2	Year 3
Daily Bookings	400	3,000	15,000
Avg Commission / Booking	INR 100	INR 100	INR 120
Annual Revenue	**INR 1.44 Cr**	**INR 10.8 Cr**	**INR 64.8 Cr**
Active Doctors	2,000	15,000	75,000
Active Patients	50,000	5,00,000	30,00,000

Market Opportunity

India's healthcare market is massive and underserved by digital solutions

TAM **INR 8.5 Lakh Cr**

Total India Healthcare Spend (2025)

SAM **INR 1.2 Lakh Cr**

OPD Consultations — Govt + Private Hospitals

SOM **INR 3,600 Cr**

DigitallyAddressableOPDMarket (Year 3 Target)

KEY NUMBERS

12 L+	Doctors in India
70K+	Hospitals (govt + private)
1.4 B	Population
850M	Smartphone users
5%	OPD currently digitized
INR 500	Avg OPD consultation fee
70%+	Rural population uses public health system

Target Users

Patients

- Rural patients needing specialist access across districts
- Urban patients seeking real-time nearby doctor availability
- Chronic-disease patients (diabetes, BP, heart) needing regular visits
- Elderly & women with mobility constraints
- Daily-wage workers who cannot afford wasted trips
- Caregivers seeking doctors for family members

Doctors

- Private practitioners with own clinics / polyclinics
- Govt hospital doctors (PHC, CHC, District)
- Specialists seeking to fill empty OPD slots
- New doctors building their patient base
- Doctors working across multiple facilities
- Retired doctors offering limited-hour consultations

Hospitals

- Govt district & sub-district hospitals
- Private hospitals & nursing homes
- Primary Health Centres (PHCs)
- Community Health Centres (CHCs)
- Multi-specialty private hospital chains
- Rural clinics and dispensaries

Government

- Ministry of Health & Family Welfare
- State health departments and directorates
- NHA (National Health Authority)
- District health officers (CMOs/DHOs)
- Ayushman Bharat Digital Mission team
- NITI Aayog health division

Competitive Landscape

Platform	Real-Time Availability	Govt Hospitals	Private Hospitals	Rural Focus	Appointment Booking	Commission Model
Practo	No	No	Yes	No	Yes	Varies
Apollo 24/7	Partial	No	Apollo only	No	Yes	In-house
eSanjeevani	No	Yes	No	Partial	Tele only	Free (Govt)
Credihealth	No	No	Yes	No	Yes	B2B
DOCVAIL	**YES**	**YES**	**YES**	**YES**	**YES**	**80/20 Split**

DOCVAIL'S UNIQUE ADVANTAGES

- ONLY platform covering both govt and private hospitals with real-time doctor availability — no competitor does this.
- Rural-first approach — designed for low-bandwidth connections, Hindi/regional language support, SMS fallback for non-smartphone users.
- Doctor-friendly 80/20 commission — highest payout ratio in the market. Practo and others charge higher platform fees.
- Government data integration via authorized API — once established, this becomes an unassailable competitive moat.

Digital India Readiness

India's digital infrastructure is ready for Docvail

850M+	800M+	75 Cr+	40 Cr+	6.5 L+
Smartphone Users	Internet Users	ABHA Health IDs	Monthly UPI Txns	CSC Centres

GOVERNMENT DIGITAL HEALTH ECOSYSTEM ALIGNMENT

- Ayushman Bharat Digital Mission (ABDM) — unified health ID for all citizens, Docvail integrates natively.
- ABHA (Ayushman Bharat Health Account) — 75 Cr+ IDs already created, enabling seamless patient identification.
- UPI payments — enables instant consultation fee collection with automatic 80/20 commission split to doctor and platform.
- Common Service Centres (CSCs) — 6.5 lakh centres across rural India provide digital access for non-smartphone patients.
- NHA Open APIs — National Health Authority provides health data APIs that Docvail can build hospital integration on.
- eSanjeevani infrastructure — existing telemedicine backbone proves govt willingness to digitize healthcare delivery.

Go-to-Market Strategy

Phase 1
M1 - M6

Foundation
- Launch in 2-3 pilot districts
- Onboard private clinics FIRST (no govt approval needed)
- Apply for govt hospital data access authorization
- Target: 200+ doctors, 5,000+ patients registered
- First revenue from commission bookings
- Iterate on product-market fit

Phase 2
M6 - M12

Govt Integration
- Secure govt authorization for hospital data
- Integrate PHCs and district hospitals
- Launch in 10+ districts across 3 states
- Target: 2,000+ doctors, 50,000+ patients
- INR 1 Cr+ ARR from commissions
- Hire on-ground onboarding teams

Phase 3
M12 - M24

Scale
- Expand to 10+ states, 50+ districts
- Onboard hospital chains and multi-specialty
- Launch premium featured doctor profiles
- Target: 15,000+ doctors, 5L+ patients
- INR 10 Cr+ ARR
- Series A fundraise readiness

Phase 4
M24 - M36

National
- Pan-India coverage — all states and UTs
- Deep govt integration (all district hospitals)
- Launch hospital analytics subscription
- Target: 75,000+ doctors, 30L+ patients
- INR 60 Cr+ ARR target

Category leadership position

Government Partnership

Seeking government support to scale real-time healthcare data access

WHAT WE SEEK FROM GOVERNMENT

1. Authorization to access government hospital doctor rosters and OPD schedules in real-time via secure API.
2. Integration support with ABDM / ABHA health ID ecosystem for seamless patient identification on Docvail.
3. Pilot deployment support and endorsement in 5 initial districts to validate the platform with real users.
4. Official recognition under Ayushman Bharat Digital Mission as a healthcare innovation partner.
5. Endorsement letter for rural healthcare digitization to support investor and hospital onboarding efforts.

WHAT GOVERNMENT GETS IN RETURN

1. Real-time nationwide visibility into doctor availability across all government hospitals and health centres.
2. Significant reduction in OPD overcrowding — patients arrive only when their doctor is available.
3. Data-driven healthcare resource allocation — identify doctor shortages and optimize postings.
4. Zero cost to government — Docvail builds, operates, and maintains the entire platform at no public expense.
5. Direct alignment with Digital Health Mission goals — measurable improvement in rural healthcare access.
6. Improved public health metrics — fewer wasted visits, better patient outcomes, higher system efficiency.

With govt data authorization, Docvail can display real-time availability of every govt hospital doctor in India.

Risks & Mitigations

Risk	Level	Mitigation Strategy
Govt data access delayed or denied	**High**	Start with private hospitals first; govt integration as Phase 2. Platform is viable without govt data.
Low doctor adoption in early stages	**Medium**	80/20 commission incentive + zero cost + dedicated on-ground onboarding team in pilot districts.
Competition from Practo, Apollo 24/7, others	**Medium**	Unique value: real-time availability + govt hospitals + rural focus. No competitor covers all three.
Payment collection challenges in rural areas	**Medium**	UPI auto-debit for digital users + cash payment option via hospital counter for rural patients.
Data privacy and regulatory compliance	**Medium**	ABDM compliance from day 1 + end-to-end encryption + minimal PII storage + regular audits.
Rural internet connectivity issues	**Medium**	Progressive Web App + SMS fallback for availability checks + offline mode + CSC centre partnerships.
Infrastructure scaling costs as user base grows	**Low**	Cloud-native auto-scaling architecture. Costs grow linearly with revenue. Positive unit economics.

The Ask

Seeking INR 50 Lakh - 1 Crore (Seed Round)

To build the MVP, launch pilot, and prove product-market fit in 3 districts

USE OF FUNDS

Category	Amount (INR)	Purpose
MVP Development	15 - 20 Lakh	Mobile app (iOS/Android) + web portal + doctor dashboard + backend infrastructure
Pilot Launch	8 - 10 Lakh	On-ground team for doctor onboarding + district-level operations in 3 pilot districts
Marketing & Growth	10 - 12 Lakh	Patient acquisition campaigns + doctor outreach + branding + social media
Operations & Runway	12 - 15 Lakh	Core team salaries + cloud infra costs + 12-month operational runway
Govt Liaison & Legal	3 - 5 Lakh	Legal compliance + data access applications + govt meeting travel + IP filing

WHAT YOU GET

- Equity stake in a first-mover platform covering India's entire hospital ecosystem (govt + private, rural + urban).
- Proven commission model with clear path to INR 10 Cr+ ARR in Year 2 and INR 60 Cr+ in Year 3.
- Government integration moat — once data access is authorized, this is extremely hard for any competitor to replicate.
- Social impact alignment — improving healthcare access for 900M+ underserved Indians. Strong ESG narrative.

Our Team

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EDUCATION :-

Integrated M.Sc. Computer Science

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Thank you